

Six burdens of Lyme disease and Long COVID that the published evidence leaves unmeasured

Policy brief. Uri Kartoun, PhD, DBbun LLC. 10 October 2026 (counts rechecked). Independent entry in the TOPx HHS Tech Sprint; not endorsed by HHS or NIH. Built from the 25 simulators in the DBbun library (12 Lyme disease, 13 Long COVID), each made from published papers.

Finding

Across the 25 simulators, 67 numeric results were tagged with one of 16 burden categories. 49 of those results (73%) fall in just three categories: work and income, direct costs of care, undercounted by official surveillance. Six categories have no tagged result in any simulator, for either condition: care continuity risk, caregiving and household labor, cognitive impact, congenital/parent-to-child transmission, longer-term opportunity costs, psychological burden. That shows where the numbers are thin. It does not show that these burdens are small.

Burden category	Lyme	Long COVID	Total
Work and income	2	15	17
Direct costs of care	13	3	16
Undercounted by official surveillance	3	13	16
Diagnostic delay	6	none	6
Barriers to reaching care	4	none	4
Overall quality of life	none	3	3
Administrative burden	2	none	2
Harm from misdiagnosis-driven treatment	1	none	1
Medical disbelief and dismissal	1	none	1
Uncoordinated, fragmented care	none	1	1
Care continuity risk	none	none	none
Caregiving and household labor	none	none	none
Cognitive impact	none	none	none
Congenital/parent-to-child transmission	none	none	none
Longer-term opportunity costs	none	none	none
Psychological burden	none	none	none

Numbers are results per burden category (each result has at most one tag). Bold rows have none.

Why it matters

- **Claims and codes miss it.** In 14 of 25 simulators the source document itself warns that claims data or ICD-10 codes undercount the condition. In 20 of 25 it describes a concrete gap in the care or evidence system itself.
- **What is counted is lopsided.** In these papers, costs, lost work and undercounting are the best-measured burdens. Caregiving, schooling, thinking, mood and the fragility of home care are not measured at all.

What measuring the six would look like

- **Care continuity risk:** Share of care that depends on one person.
- **Caregiving and household labor:** Caregiver hours per week.
- **Cognitive impact:** Task completion time; share needing help.
- **Congenital/parent-to-child transmission:** Transmission rate; number of children affected.
- **Longer-term opportunity costs:** Years of schooling disrupted; tutoring hours; projected career earnings lost.
- **Psychological burden:** Anxiety or depression prevalence.

Suggested next steps (the author's suggestions, not findings)

- Ask whether an existing federal survey could add one item for each of the six, starting with caregiver hours per week and years of schooling disrupted.
- Run any new paper through the DBbun pipeline (documented API, Python or Excel) to see which of the 16 burdens it measures and which it leaves open.

How far to trust this

The papers were chosen by DBbun, not through a systematic review, so the counts describe this library, not all Lyme disease or Long COVID research. An AI model (Claude Sonnet 5, claude-sonnet-5) assigned each tag; code checked them against a fixed list. "None" means no quantified result in these papers carried the tag, not that the burden is absent or was never studied elsewhere. No patient data was used. The brief was drafted with Claude Code (Claude Sonnet 5.5, claude-sonnet-5-5). Live counts: public demo, Simulation library.